

1. Administrative & Study Identification

Full Study Title: A Study to Evaluate the Safety, Tolerability, Drug Levels, and Preliminary Efficacy of BMS-986507 Combinations in Adult Participants With Advanced Solid Tumors **Short Title/ Acronym:** CA244-0001 / BMS-986507 Combinations Study **Protocol Number:** CA244-0001 **NCT Number:** NCT06618287 **Posting ID:** 637183924785060484 **Sponsor/Company Name:** Bristol Myers Squibb **Investigational Product:** BMS-986507 (iza-bren / izarontamab brengitecan) in combination with Osimertinib, Pembrolizumab, Nivolumab, or Pumitamid **Phase of Development:** PHASE1, PHASE2 (Phase 1/2a) **Trial Status:** RECRUITING **Medical Monitor:** Bristol Myers Squibb Clinical Operations Lead

2. Study Synopsis & Rationale

2.1 Study Objective * **Primary Objective:** To evaluate the safety, tolerability, drug levels, and preliminary efficacy of BMS-986507 (BL-B01D1) combinations in adult participants with advanced solid tumors. * **Secondary Objectives:** To assess objective response rate (ORR), best overall response (BOR) per RECIST v1.1, and pharmacokinetics (PK, such as Cmax, Tmax, AUC).

2.2 Background & Rationale To address the unmet medical need in patients with advanced solid tumors, including non-small cell lung cancer (NSCLC) and triple-negative breast cancer (TNBC) or ER-low/HER2-negative breast cancer. BMS-986507 is a potential first-in-class antibody-drug conjugate (ADC) that has shown promising clinical activity. This study will identify the Maximum Tolerated Dose (MTD)/Recommended Phase 2 Dose (RP2D) and evaluate preliminary efficacy when combined with standard targeted therapies and immunotherapies.

3. Study Design & Methodology

3.1 Design Framework * **Study Type:** Interventional * **Control Type:** Multi-cohort (Single Group Assignment per cohort) * **Blinding:** Open-label (None) * **Randomization:** Randomized (Umbrella/Platform assignment into specific targeted groups)

3.2 Planned Enrollment & Duration * **Targeted Enrollment (\$N\$):** 238 participants * **Number of Sites:** Approximately 63 locations * **Estimated Study Start:** Q4 2024 * **Current Status:** Recruiting

4. Subject Selection (Eligibility Criteria)

4.1 Inclusion Criteria 1. Age $\geq$ 18 years. 2. Participants must have at least one measurable lesion per response evaluation criteria in solid tumors (RECIST v1.1). 3. Eastern Cooperative Oncology Group (ECOG) Performance Status of 0 or 1. 4. Life expectancy of at least 3 months at the time of the first dose. 5. Disease-specific cohorts: * **Group A:** Pathologically confirmed locally advanced or metastatic NSCLC with an EGFR exon 19 deletion or L858R mutation in exon 21, either alone or in combination with other EGFR mutations, which may include T790M in exon 20. Participants with other EGFR mutations (including but not limited to, exon 21 L861Q, exon 18 G719X, and exon 20 S768I mutations, etc.) will also be allowed. * **Group B:** Pathologically confirmed locally advanced or metastatic NSCLC. * **Group C:** Pathologically confirmed locally-advanced, recurrent inoperable, or metastatic TNBC or ER-low, HER2-negative BC. * **Group D:** Pathologically confirmed locally-advanced, recurrent inoperable, or metastatic TNBC. * **Group E:** Pathologically confirmed locally advanced or metastatic NSCLC, not amenable to treatment in curative intent.

4.2 Exclusion Criteria 1. Participants must not have any mixed Small Cell Lung Cancer (SCLC) and Non-Small Cell Lung Cancer (NSCLC) histology. 2. Untreated symptomatic central nervous system (CNS) metastases. 3. Participants with known mutations in EGFR will be excluded (for Group B). 4. Participants must not have a history of serious recurrent infections. 5. Participants must not have a history of severe heart disease. 6. Grade 3 lung disease or a history of interstitial lung disease.

5. Intervention & Treatment Plan

5.1 Investigational Regimen * **Dosage/Frequency:** BMS-986507 administered at specified dose levels on specified days during 21- or 28-day cycles (dose-finding and expansion phases). * **Combinations:** * Group A: BMS-986507 + Osimertinib * Group B: BMS-986507 + Pembrolizumab * Group C: BMS-986507 + Nivolumab (Trade Names: Opdualag, Opdivo | ATC Code: L01FF01) * Group D & E: BMS-986507 + Pumitamid * **Route of Administration:** Intravenous (for BMS-986507, Pembrolizumab, Nivolumab, Pumitamid); Oral (for Osimertinib). * **Duration of Treatment:** Continued until disease progression, unacceptable toxicity, or patient withdrawal.

5.2 Concomitant & Prohibited Therapy * **Permitted:** Standard supportive oncology care. * **Prohibited:** Other concurrent investigational agents or unapproved anticancer therapies.

6. Study Timeline & Visit Schedule

Visit ID	Window (Days)	Key Activities/Procedures
Screening	Day -28 to 0	Informed Consent, Medical History, Tumor Mutational Profiling, Baseline Imaging
Baseline	Day 0	Cohort Assignment, First Dose of combination therapy, Safety Baseline
Interim	Every Cycle	PK/PD Labs, Safety Monitoring, DLT Evaluation (first cycle)
Tumor Assessment	Q6-8 Weeks	Imaging (CT/MRI) for RECIST v1.1 evaluation
End of Study	Follow-up (30-100 days post-last dose)	Safety follow-up, Survival tracking

7. Outcome Measures (Endpoints)

7.1 Primary Endpoint * Number of participants with adverse events (AEs). * Number of participants with serious adverse events (SAEs). * Number of participants with AEs meeting protocol-defined dose-limiting toxicity (DLT) criteria.

7.2 Secondary & Exploratory Endpoints * Pharmacokinetics: * Maximum observed serum concentration (C_{max}) * Time of maximum observed concentration (T_{max}) * Area under the concentration-time curve from time zero to time of last quantifiable concentration (AUC(0-T)) * Objective Response Rate (ORR) and Best Overall Response (BOR) assessed by investigator per RECIST v1.1. * Progression-Free Survival (PFS).

8. Safety & Adverse Event Reporting

- **Adverse Event (AE) Monitoring:** Continuous monitoring during the DLT evaluation period and throughout treatment cycles.
 - **Serious Adverse Events (SAEs):** Reported within 24 hours to the Sponsor/Regulatory bodies.
 - **Data Monitoring Committee (DMC):** Internal safety review committee utilized to govern dose-escalation decisions and cohort expansion.
-

9. Statistical Analysis Plan (SAP) Summary

- **Analysis Populations:** All Enrolled Subjects (Safety Set) for primary safety/DLT endpoints; Efficacy Evaluable Set for ORR/PFS.
- **Sample Size Justification:** Targeted enrollment is 238 participants, based on standard Phase 1/2 dose-escalation and expansion rules to establish a safety profile and preliminary efficacy signal.
- **Handling of Missing Data:** Censoring applied for survival and progression endpoints as per standard oncological trial parameters.

10. Quality Assurance & Regulatory Compliance

- **GCP Compliance:** This study will be conducted in accordance with Good Clinical Practice (GCP) and the Declaration of Helsinki.
 - **Monitoring Plan:** Regular onsite and centralized remote data monitoring.
 - **Audit Trail:** Validated EDC systems (eCRF) will be used to track clinical responses, RECIST assessments, and DLTs.
-

11. Study Identifiers & Governance

- **Primary ID:** CA244-0001
 - **Registry Number:** NCT06618287
 - **Posting ID:** 637183924785060484
 - **Sponsor/Lead Organization:** Bristol Myers Squibb
 - **Trial Status:** RECRUITING
 - **Study Title:** A Study to Evaluate the Safety, Tolerability, Drug Levels, and Preliminary Efficacy of BMS-986507 Combinations in Adult Participants With Advanced Solid Tumors
 - **Official Regulatory Title:** A Phase 1/2a, Open-label, Dose-finding Study to Evaluate the Safety, Tolerability, Pharmacokinetics and Preliminary Efficacy of BMS-986507 (BL-B01D1) Combinations in Adult Participants With Advanced Solid Tumors
-

12. Clinical Framework (Oncology Specific)

- **Disease Areas:** Lung Cancer, Breast Cancer
 - **Primary Condition:** Advanced Solid Tumors (Malignant neoplasm of breast, NSCLC, TNBC)
 - **Diagnosis Threshold:** Histologically or cytologically confirmed locally advanced or metastatic disease
 - **Medical Methodology:** Targeted therapy + Antibody-Drug Conjugate (ADC) / Immunotherapy combinations
 - **Optimization Target:** Evaluation of Maximum Tolerated Dose (MTD), safety profile, and RECIST v1.1 tumor response
-

13. Study Procedures & Methodology

- **Management Pathway:** Standard clinical pathway for advanced solid malignancies
- **Education Protocol (HCP):** DLT identification, AE management (particularly ILD/pneumonitis tracking for ADCs), RECIST measurement protocols
- **Education Protocol (Patient):** Informed consent, reporting of immune-related or infusion-related side effects

- **Quality Audit Mechanism:** Routine site initiation, source data verification (SDV), and RECIST independent/investigator review checks

- **Total Duration:** Treatment until progression or unacceptable toxicity; survival follow-up indefinitely
- **Onsite Visit Cadence:** Day 1 of every treatment cycle (typically 21-28 days)
- **Remote Monitoring Frequency:** Continuous remote AE tracking and safety lab verification
- **Checklist Review Interval:** Tumor imaging scans every 6 to 8 weeks to determine objective response

Role	Name	Signature	Date		:---	:---	:---	:---
Principal Investigator	[Name]	_____	[DD-MMM-YYYY]					
Clinical Operations Lead	[Name]	_____	[DD-MMM-YYYY]					Lead
Statistician	[Name]	_____	[DD-MMM-YYYY]					